

Letter of instruction — psychiatric report

Dr Ravikumar Bangalore Krishnaiah, Consultant Psychiatrist, Mind and Memory Service

From: Cory Lea Shepherd · WC/2024/227, Queensland Industrial Relations Commission · [DATE]

PART A — THE ATTACHMENTS

Four documents accompany this letter. **Three of the four were written by the other parties to my matter — the Workers' Compensation Regulator, and my employer.**

No.	Document	Author	Status
1	Response of the Workers' Compensation Regulator , signed 18 February 2026 — a document in which the Regulator states its position, item by item, on a list of factual matters.	Ms Renee Matheson Senior Appeals Officer, Workers' Compensation Regulator	Assumed fact
2	Letter of the Chief Executive , Metro South Hospital and Health Service, 5 June 2026 (ref K-LM26/729), to the Commission.	The employer's Chief Executive	Assumed fact
3	Role description — Administration Officer, Switchboard Services (AO3), Logan Hospital.	The employer	Assumed fact
4	The clinical record — your own file for me from 24 October 2024; my general-practice records (Our Medical Ashmore); the workers' compensation medical certificates; my certificate of capacity of 3 July 2026.	Clinicians	Clinical record (not assumed fact)

Also enclosed, and not evidence: a **schedule of assumed facts**, which identifies by source and paragraph the facts referred to in Part B and records what the Regulator does *not* accept; and a copy of the **Notice of Non-Party Disclosure** served on your practice in this proceeding. **The four attachments themselves are provided complete and unmarked.**

PART B — THE BASIS ON WHICH I ASK YOU TO WRITE

I ask you to reason from the following, and from nothing else in relation to the workplace.

	Reason from	Where
1	The facts accepted by the Workers' Compensation Regulator , in the response signed by Ms Renee Matheson, Senior Appeals Officer , on 18 February 2026	Attachment 1
2	The letter of the Chief Executive of Metro South Hospital and Health Service , 5 June 2026	Attachment 2
3	The requirements of the position , as my employer describes them	Attachment 3
4	The clinical records, in full — your own file, my general-practice records and the certificates	Attachment 4
	And take into account	Asked at
5	The relationship breakdown and the other life stressors recorded in your own file	question 3.1
6	The past medical history recorded in the general-practice records	question 1.3

Items 1 to 3 were written by the other parties to my matter, and I ask you to assume the facts they record. **My own account of the workplace is provided as clinical context only, and is not the basis on which I ask you to reason.** Where your history from me differs from any fact in Attachments 1 to 3, please say so expressly rather than resolve the difference.

The schedule also records the matters the Regulator does not accept. Please do not treat any of those as established. They are set out so that you have the position complete rather than one side of it.

Please do not adopt, or treat as established, any conclusion, characterisation or finding of any other decision-maker. **Your opinion is yours alone.**

The sequence, from a source other than my account. The following dates are taken from the Regulator's amended List of Documents dated 14 August 2026, by its item number.

Item	Date	Document
46	1 July 2024	Text, WorkCover to me, noting the claim registered
7	1 July 2024 11 August 2024 8 September 2024	Workers' compensation medical certificates — Dr Peter Hawes
8	7 August 2024	Workers' compensation medical certificate — Dr Ki Pang
2	13 September 2024	WorkCover Queensland reasons for decision
3	16 September 2024	Application for review
9	24 October 2024	Email, Dr Krishnaiah — noting injury and medication
4	24 October 2024	Review Unit reasons for decision
5	26 November 2024	Notice of Appeal
10	13 February 2025	Report of Mind and Memory Service
11	Various	Practice records — Our Medical Ashmore

The clinical records are the complete records held by the Regulator in this matter, provided in full and unedited — including entries unrelated to this matter and entries that may not assist me. I have not selected among them. **If any further records or other material would assist you, please say so and I will obtain them.**

PART B2 — WHAT IS ALREADY RECORDED, AND WHAT I AM NOT ASKING YOU TO ESTABLISH

Attachments 1 to 3 already record the nature of the work, what occurred, and what the employer says was and was not in place. **You are not asked to find any of that, and you are not asked to take any of it from me.**

Already recorded, by the Regulator or by my employer: the requirements of the position · the accuracy of Switchboard information as critical to clinical handover and patient safety · the applicable minimum break and the 7-hour break in fact rostered on 17 to 18 March 2024, described as human error · the related administrative sequence in rostering, leave and pay · and that **no fatigue risk assessment applied to the position**, fatigue risk management being implemented only after 30 June 2024, with no consequential changes to operating procedures. **The enclosed schedule sets each out by source; you are not asked to find any of them.**

One further matter, provided for completeness. In May 2024 I made a complaint concerning the conduct of my line manager and the Director. On **24 December 2024** the Ethical Standards Unit determined that the complaint **constituted a Public Interest Disclosure**. That fact is accepted by the Workers' Compensation Regulator. **I provide it as part of the occupational history and for no other purpose.** I do not ask you to express any view on the merits of the complaint, on anything that followed it, or on any connection between it and any later event. **If it does not bear on your clinical opinion, please say so.**

What remains, and what I do ask you, is the clinical question: whether exposure of that kind bears on the condition you have diagnosed.

PART C — THE MATTERS IN ISSUE

You have been served with a Notice of Non-Party Disclosure in this proceeding. That notice states the matters in issue as follows:

- “1. Did Mr Shepherd sustain a personal injury*
- 2. Did the personal injury arise out of or in the course of Mr Shepherd's employment*
- 3. Was Mr Shepherd's employment a significant contributing factor to the injury”*

I ask you to address those three matters, in that order. My employer has separately asked for medical information about my capacity for work; **those matters are taken up within matter 3 below**, so that one report answers both.

MATTER 1 — DID MR SHEPHERD SUSTAIN A PERSONAL INJURY?

1.1 The diagnosis; the classificatory framework applied and the code; the specific diagnostic criteria met and when each was met; and the date of onset as best the clinical record establishes it.

1.2 The differential diagnoses considered, and your reasons for including or excluding each.

1.3 My background. I ask you to reason from the following rather than around it.

(a) The records include a referral letter of **16 May 2024, Dr William Zhao to Dr Arash Amini**, listing under “*Past Medical History*”: **26/10/2022 ADHD · 26/10/2022 Anxiety · 28/06/2023 Infection**. In respect of each, is it a **diagnosis made at a consultation, or a history-list item carried forward? Is any anxiety disorder diagnosed anywhere in the material?**

(b) What do the **current medications recorded in that same letter** indicate about whether any psychiatric condition was being treated as at **16 May 2024?**

(c) Is attention deficit **a longstanding neurodevelopmental condition rather than a psychiatric injury** — and is a person with those features **more, or less, vulnerable to injury where continuous shift work is worked without adequate recovery between shifts**, and under sustained high-volume concurrent demand of the kind at Attachment 3?

(d) Dr Priyal De Silva Nanayakkara recorded on **16 November 2023**: “*Poor sleep. Shift work. Takes melatonin 2 mg to help to go to sleep, soem time 4 mg, but does not help much. Cannot work/ do shifts if he does not get a good sleep. Does not take them every day. No psychological illness such as depression/ psychosis. mood good.*” — prescribing melatonin and temazepam. What does that entry indicate about **psychiatric illness at that date**, and about **sleep disturbance attributed to shift work** seven months before onset?

(e) The consultation note of **16 May 2024** records “*renew referral to psychiatrist*”, and the letter at (a) is that referral — “*for ongoing care and management*” — **one month before the date of onset**. What significance do you attach to it, given that the record of 16 November 2023 states “*No psychological illness ... mood good*” — and **what, if anything, does the material show occurred between those two dates?** When did psychiatric care begin, and for what?

(f) Please identify **each place anxiety is recorded, by author and date**, and in each case whether it is a **presenting symptom or a diagnosis** — including **28 June 2024**, recorded by Dr Bogdan Slawinski, where the presentation is “*wants melatonin, to help to sleep ... stress at work ... upset by people not following rules*” and the reason for visit “*Anxiety*”.

(g) Dr Peter Hawes recorded on 1 July 2024 that I had “*been there 5 years*”; I worked continuous shift work in this position from 2019. What does the material show as to **any psychiatric treatment, or any absence from work on psychological grounds**, before 2024 — and **what does that indicate about my level of function in this role?** Does anything in the records bear on the date of onset?

(h) If there was a pre-existing condition or vulnerability, did the matters at Attachments 1 to 3 **aggravate** it, and to what extent?

1.4 Whether any premorbid personality features bear on the diagnosis, and if so how — from your own current clinical assessment. I do not ask you to adopt or repeat any earlier characterisation.

1.5 A complete list of every document you reviewed and every person you saw, with the dates and duration of each examination.

MATTER 2 — DID THE INJURY ARISE OUT OF, OR IN THE COURSE OF, THE EMPLOYMENT?

2.1 The conditions of the work. Attachment 3 records that the position requires continuous shift work across the full 24-hour period, seven days a week; participation in the Emergency Response process, distributing emergency notifications in accordance with code procedures and “*strictly adhering to protocols and timeframes*”; “*maintaining call queues to minimum at all times*”; multitasking and operating under pressure with “*high volume call traffic*”; exercising judgement “*in situations where precedence have not been set and procedures not defined*”; and working with “*limited supervision*”. Attachment 1 records that the accuracy of Switchboard information is **critical to clinical handover and patient safety**.

From a clinical perspective, whether and how conditions of that kind — the level of demand, the degree of control, and the adequacy of support — can contribute to a condition of the kind you have diagnosed.

2.2 Fatigue, and the rostering of 17 to 18 March 2024. Attachment 1 records that on those dates I was rostered to finish at 23:00 and to commence the next shift at 06:00 — **a break of 7 hours**. The applicable minimum is recorded as **10 hours, or 8 hours by written agreement**. A further document of the Regulator, dated 13 May 2026, records that an 8-hour written agreement applied and describes the shift as separated “by only a 7-hour break (rather than an 8-hour break) ... a result of human error”. **On either figure the break was shorter than the minimum**. Attachment 2 records that emergency (MET) calls were recorded on those shifts, and that leave was taken on 19 March 2024. Attachment 2 also records that **no fatigue risk assessment applied to the position**, and that fatigue risk management was implemented at that Switchboard only after 30 June 2024.

What clinical significance, if any, do you attach to that sequence — and does fatigue arising from a shortened break resolve with the break itself, or can it carry forward?

2.3 The administrative sequence. Attachment 1 and the further document of 13 May 2026 also record: an email of the Director dated **7 August 2023** acknowledging “a rostering error that was accidentally made ... with regards to night shifts”; that a leave application for **20 to 27 February 2024** was declined for want of an attached statutory declaration when “a review indicates that in fact, the attachments were present”, described as “human error”; and that a payroll instruction to correct my shifts was issued on **3 May 2024** and the correction submitted on **28 May 2024**, with an email of 21 May 2024 in the interval.

From a clinical perspective, does a sequence of that kind — repeated administrative error affecting rostering, leave and pay, in a role for which no fatigue assessment applied — bear differently on the condition you have diagnosed than a single isolated event would? Please explain the mechanism, if any.

2.4 The three stressors identified in the capability checklist. The employee capability checklist of 3 July 2026 identifies symptom exacerbation on exposure to **(i) complaint handling, (ii) being held accountable and blamed for the failures of others, and (iii) unpredictable rostering**. **Please state the clinical basis on which each of those three is identified as contributing to the condition**, by reference to the material. As to (ii), the assumed facts include: a rostering error acknowledged in writing as having been made by another person; a leave application declined for a missing attachment which a review found “in fact... present”; and a payroll correction which another person was instructed to make and which took 25 days. **Whether bearing the consequence of errors made by others is a recognised occupational stressor, and by what mechanism.**

MATTER 3 — WAS THE EMPLOYMENT A SIGNIFICANT CONTRIBUTING FACTOR?

This matter has three parts, and I ask you to take them in the order given — the other factors and their timing first, then the interval and the mechanism, and your conclusion last, so that the conclusion rests on the two answers before it. The questions on capacity, restrictions and adjustments follow at 3.4 to 3.8.

3.1 Other factors, placed in time. Please identify any non-employment factors relevant to **(i) onset at or about 18 June 2024** and **(ii) the subsequent course**. Your report of 13 February 2025 referred to multiple life stressors including relationship breakdown, job loss and bereavement.

Your clinical relationship with me began on 24 October 2024, and my partner attended that consultation with me. Each of the matters above arose while I was under your care and is recorded in your own file. I ask you:

(a) to place each factor in time from your own records rather than by assumption, and to state for each whether it arose before or after 18 June 2024;

(b) to state, for any factor arising after that date, whether it is capable of explaining the onset; and to state whether, **as at the date of your diagnosis**, my relationship was a protective factor or an adverse one; and

(c) for each factor, whether in your opinion it was independent of the employment matters, or a consequence of them — noting that your report of 13 February 2025 records that my pay was “withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship”.

3.2 The interval, and the mechanism. The rostering and administrative matters at 2.2 and 2.3 occurred between August 2023 and May 2024, and the date of onset in this matter is **18 June 2024**. Is that interval

consistent with, or inconsistent with, your formulation — and if consistent, **by what mechanism?**

Separately, what significance, if any, attaches to the interval between onset and first presentation, the first workers' compensation medical certificate in the List of Documents being dated 1 July 2024? Please take into account your answer at **1.3(d)** as to the renewal of psychiatric referral on 16 May 2024.

3.3 Causation — your conclusion. On the assumed facts, and having regard to your answers at 3.1 and 3.2, whether my employment was **a significant contributing factor** to the injury with onset on 18 June 2024. Please give your clinical reasoning, and distinguish between the cause of onset and any factors bearing only on the subsequent course. **I do not ask you to find that employment was the only cause, or the main one** — only whether, on the material, it was a significant contributing factor.

Capacity, restrictions and adjustments — the matters my employer has asked about. My certificate of capacity of 3 July 2026 certifies me fit for my substantive role with adjustments; I ask only that your opinion be your own.

3.4 Prognosis and current capacity. Your prognosis; and, distinguishing **(a)** my capacity to perform the substantive role **with reasonable adjustments** from **(b)** any current incapacity. If there is current incapacity, its cause — the condition itself, the consequences of my having been away from the workplace since 3 July 2026 (a matter of my account, not of Attachments 1 to 3), or other factors.

The functional record is already in Attachment 4 — the employee capability checklist of 3 July 2026 sets out the pattern of work actually performed and the restrictions then in place. **You may take that record as established and need not reconstruct it.** What I ask for at 3.5 and 3.6 is your own opinion.

3.5 The adjustments, in functional terms. The adjustments you consider clinically necessary, the clinical basis for each, and the anticipated duration and review date — addressing, among anything else you consider relevant: predictability of rostering; adequate recovery between shifts; sustained concurrent emergency-code load; hours of work; and reporting arrangements. Please also address, in functional terms:

(a) any tasks, situations or environments likely to **exacerbate** the condition;

(b) if you consider a restriction on **complaint handling** clinically necessary, what activities it encompasses — receiving, documenting, redirecting or resolving complaints, or all complaint-related interaction. **My employer records that the established process for client complaints received by Switchboard employees is for immediate escalation to, and management by, the Client Liaison Officer and/or the Manager, Switchboard Services.** Please frame the restriction by reference to that process; and

(c) your report of 13 February 2025 records that my **working memory** is affected under stress — what that means for performance of the duties at Attachment 3, and the circumstances likely to give rise to it.

3.6 The requirements of the position. Attachment 3 describes the duties of the position and states that it is a continuous shift working role. Please address, in order:

(a) whether the adjustments at 3.5 are **clinically necessary at present** — that is, whether I am presently able to perform the duties without them. **If your answer is that I am not, please identify which duties and why**, so that the answer is not read as a general incapacity for the role;

(b) whether, **with those adjustments in place**, I am able to perform the duties described at Attachment 3;

(c) whether adjustments of that kind — predictability of rostering, minimum recovery intervals between shifts, limits on sustained concurrent emergency-code load, and hours of work — are **of a kind ordinarily provided by large employers, and in particular by health services operating 24-hour services**; and

(d) whether the adjustments are **permanent, or temporary with a review date**.

I note that the employee capability checklist of **3 July 2026** at Attachment 4 records: *“This reduced pattern of approximately **six shifts per fortnight** is the pattern Mr Shepherd **has in fact worked and tolerated over the past twelve months without deterioration**, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation. Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.”* Please take that record into account in answering (b) and (c).

3.7 Directions and workplace discussions. Whether there is any clinical impediment to my **following a reasonable and lawful direction** given by a supervisor, or to my **participating in discussions concerning workplace performance or conduct**. If any adjustment would be clinically indicated for discussions of that kind — for example notice, a written agenda, the presence of a support person, or duration — please specify it and its clinical basis.

3.8 Foreseeable risk. Whether exposure to the conditions described at Attachments 2 and 3, **without** the adjustments at 3.5 and in the absence of a fatigue risk assessment, presents a foreseeable risk to my health or safety; and if so, what controls you consider medically necessary to manage it.

PART D — WHAT I DO NOT ASK, AND THE FORM OF THE REPORT

I do not ask you to express a view on whether any management action was reasonable; on whether my employer is able to accommodate the adjustments you recommend; or on my capacity to work under any particular reporting line or with any particular person. **Those are workplace questions, not medical ones. If you are asked any of them, I ask that you say so rather than answer.**

As to form, I ask that the report:

- (a) answer under the numbering used in this letter**, so that each question and its answer can be identified — and where a question is not answered, say so under its number and why;
- (b) state your qualifications and specialty**, and the dates and duration of each consultation on which your opinion is based;
- (c) reason from the assumed facts to your conclusions rather than assert conclusions**, and state where the material is insufficient for you to express an opinion rather than qualifying an opinion you would not otherwise give;
- (d) state expressly which of the sources at Part B your opinion as to causation rests on, and whether that opinion depends on my account of the workplace matters;**
- (e) state whether anything in the clinical records provided is inconsistent with, or qualifies, the opinions you express — and if so, how you have taken it into account**, so that the report addresses the whole of the material rather than part of it; and
- (f) confirm that the opinions expressed are your own.**

If it would assist, I am content to be provided with a draft so that I can identify any factual error or omission in the material. I will not comment on your opinions, and any correction I offer will be to the facts only.

As to who will receive the report. It will be provided to both parties to my matter, and to no one else:

- (i) to the Workers' Compensation Regulator**, by way of continuing disclosure in the Commission proceeding — the same proceeding in which the Notice of Non-Party Disclosure was served on your practice; and
- (ii) to Metro South Hospital and Health Service**, in answer to its request for medical information of 31 July 2026.

Both will receive the identical report, on the same day, together with this letter of instruction and the schedule of assumed facts — and each will be told that the other holds it. There will be no separate or edited version.

As to cost and instruction: the cost of the report is being met by **Metro South Hospital and Health Service**, which has requested medical information about my capacity for work. **The report is nonetheless instructed by me, and I ask that it be addressed to me.** Neither bears in any way on the opinion you express.

APPENDIX — THE EMPLOYER'S REQUEST OF 31 JULY 2026, AND WHERE EACH QUESTION IS ANSWERED

My employer's request is set out below in its own terms, with the question in this letter that answers it. **The full request can be provided if you wish to see it.** Where a question is not answered, the reason is that it goes to workplace arrangements rather than to medical capacity — I ask only that you say so.

Their question	Answered at
1(a) When was Mr Shepherd first diagnosed with MDD?	1.1
1(b) What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition?	2.1 · 2.2 · 2.3 · 2.4 and 3.3
1(c) Whether these workplace stressors are based solely on Mr Shepherd's self-report, your own clinical assessment, or other medical information/reports available to you.	Part B and Part D(d)
1(d) Whether exposure to these workplace stressors presents a foreseeable risk to Mr Shepherd's health or safety if he were to return to work, and if so what specific workplace controls or adjustments you consider medically necessary to manage that risk.	3.8
2 Whether Mr Shepherd is able to follow a reasonable and lawful direction issued by his supervisor; and to participate in discussions in relation to his workplace performance and/or conduct.	3.7
3 Whether Mr Shepherd is medically fit to return to his substantive role under the existing reporting arrangements, including working with and reporting to his current line manager.	Not asked. A question about workplace arrangements — please say so rather than answer
4 If not, specify any restrictions or adjustments you recommend, the clinical basis in functional terms, and the anticipated duration and review date.	3.5
5 What specific activities are encompassed within “complaint handling” — receiving, documenting, redirecting, resolving, or all complaint-related interactions.	3.5(b)
6 Is Mr Shepherd able to fulfil the full inherent requirements of his role without restrictions or modifications to duties?	3.6(a)–(d)
7 What does “working memory is affected under stress” mean in functional terms — how it may affect capacity to perform the substantive duties under normal workplace demands, and the circumstances likely to cause the difficulty.	3.5(c)
8 Are there specific tasks, situations or environments that may exacerbate Mr Shepherd's condition or symptoms?	3.5(a)
9 If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?	Not asked. A question about workplace arrangements — please say so rather than answer